Glossary of PBM Terms

Cut through the jargon. Make better decisions.

EDITION 2026.1 · JULY 2026 · 95 TERMS

PURPOSE	Plain-language definitions for the terms that appear in pharmacy benefit contracts, including the ones that carry financial consequences
AUDIENCE	Board members, plan fiduciaries, benefits leaders, and the advisors who support them
SOURCING	Contract and scoring terms are defined as they are used in the Nautilus model contract language and the CAA 2026 Readiness Report, so this glossary agrees with the rest of the project
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Pharmacy benefit contracts are filled with terms that sound technical, but many are financial traps in disguise. This glossary decodes the language PBMs use to justify spread pricing, hide rebates, or limit oversight. Whether you're a board member, fiduciary, or benefits leader, knowing these terms is your first line of defense.

If you can’t define it, you can’t manage it, and you certainly can’t negotiate it.

Numerals

- **340B Program:** A federal program requiring drug manufacturers to offer discounted prices to certain safety-net providers. Where a plan’s claims run through a 340B-eligible entity, the discount does not automatically reach the plan.

A

- **ACA (Affordable Care Act):** 2010 healthcare reform law aimed at increasing coverage and reducing costs.
- **Adverse Benefit Determination:** Denial, reduction, or termination of a benefit.
- **Alternative Funding Plan:** Program to reduce prescription drug costs through nontraditional funding mechanisms, such as manufacturer assistance programs, international sourcing, or charitable organizations.
- **Audit Rights:** The plan’s contractual right to verify what it was charged, on its own initiative. What matters is the scope: who selects the auditor, what records are reachable, whether findings can be extrapolated, and how often. A right to audit that the PBM controls is not a right to audit.
- **AWP (Average Wholesale Price):** A benchmark price for drugs published by commercial compendia; often significantly higher than actual acquisition cost.

B

- **Benchmarking:** Comparing a plan’s costs, performance, and features to industry standards or peers.
- **Beneficiary:** Person or entity designated to receive plan benefits.

- **Biosimilar:** A biologic highly similar to an approved reference product, with no clinically meaningful differences. Biosimilars often carry lower list prices and smaller rebates, which is why a rebate-driven formulary may not prefer them.
- **Brand Drug:** Medication marketed under a proprietary, trademark-protected name by its manufacturer.
- **Breach of Fiduciary Duty:** Failure to fulfill fiduciary duties such as not acting solely in the best interests of plan participants and beneficiaries.
- **Brown Bagging:** Dispensing a specialty drug to the patient, who carries it to the site of care. Cheaper than buy-and-bill, and restricted or prohibited by some provider contracts.

C

- **Carve-Out:** Separating the pharmacy benefit from the medical carrier or third-party administrator and contracting it independently, or removing a category such as specialty drugs from the main PBM agreement. Contracts that penalize or prohibit a carve-out limit the plan's ability to adopt a better option later.
- **Claims Administrator:** Manages the claims process on behalf of a plan.
- **Claims Fiduciary:** Individual or entity responsible for making final decisions on benefit claims.
- **Claims Procedure:** Process for filing and appealing benefit claims under a plan.
- **Clawbacks:** Money recovered by the PBM after a claim is already paid. Two forms matter. A *copay clawback* occurs when a participant pays more in copay than the drug costs and the PBM keeps the difference. A *post-adjudication clawback* reduces what the pharmacy was paid after the fact. A contract that permits either lets the stated price change after the plan has agreed to it.
- **COBRA:** Law providing continued health coverage after qualifying events like job loss.
- **Conflict of Interest:** Situation where personal or corporate interests may interfere with fiduciary responsibilities.
- **Consolidated Appropriations Act (CAA):** Federal law addressing transparency, surprise billing, and patient protections for employer-sponsored health plans. The 2021 act sharpened ERISA's focus on health plans, requiring compensation disclosure and prohibiting gag clauses. The 2026 requirements extend those obligations further. Together they are the basis on which pharmacy benefit contracts are now measured.
- **Copay Accumulator Program:** A program that excludes manufacturer copay assistance from counting toward a member's deductible or out-of-pocket maximum.
- **Copay Maximizer Program:** A program that spreads manufacturer copay assistance across the plan year to capture as much of it as possible, rather than excluding it from the deductible as an accumulator does.
- **Cost-Sharing:** Participant-paid healthcare costs, such as deductibles, copayments, and coinsurance.
- **Covered Entity:** Health plan, healthcare provider, or clearinghouse subject to HIPAA rules.
- **Cross-Plan Offsetting:** Using overpayments from one plan to offset payments to providers under another plan.

D

- **Data Sovereignty:** Whether the plan owns its claims and pricing data and holds the control to act on it, during the contract term and after it ends. Being shown a report is not the same as holding the data. A plan that cannot extract its data, or cannot leave with it, is renting visibility it should own.

- **Deductible:** Amount participants must pay before the plan begins to cover costs.
- **Dependent:** Spouse or child eligible for coverage under a participant's health plan.
- **DIR Fees (Direct and Indirect Remuneration):** Adjustments applied to pharmacy reimbursement after a claim is paid. Originating in Medicare Part D, the term now describes retroactive fees more broadly. They make the price at the point of sale provisional rather than final.
- **Discretionary Authority:** Decision-making power over plan management or assets, triggering fiduciary responsibilities.
- **Dispensing Fee:** A flat fee paid to a pharmacy for filling a prescription, separate from drug ingredient cost.
- **DOL (Department of Labor):** The federal agency enforcing ERISA compliance.

E

- **Effective Rate Guarantee:** A discount promised on average across a period and a book of claims, rather than on each claim. Averaging lets a shortfall on one drug be offset by a surplus on another, so a guarantee can be met while individual claims are priced poorly.
- **EGWP (Employer Group Waiver Plan):** A Medicare Part D plan offered by employers to their retirees.
- **ERISA (Employee Retirement Income Security Act of 1974):** Federal law governing employer-sponsored retirement and health plans.
- **Exclusive Benefit Rule:** Requires fiduciaries to act solely in the interest of participants and beneficiaries, for the exclusive purpose of providing benefits and defraying reasonable plan expenses.

F

- **Fiduciary:** Person or entity with discretionary authority over plan assets or administration.
- **Fiduciary Alignment:** Whether the contract supports the plan sponsor's obligation to act in the interest of participants and to see that their costs are reasonable. A contract is aligned when the PBM's conduct serves the plan, the plan's money remains a plan asset, and the sponsor holds the control to verify and enforce both.
- **Fiduciary Alignment Score (FAS):** A score from 0 to 100 measuring how closely a contract's language matches published model contract language across ten provisions in three domains: Fiduciary Conduct, Financial Integrity, and Oversight and Control.
- **Fiduciary Breach:** Violation of fiduciary duties.
- **Form 5500:** Annual report filed with the DOL for employee benefit plans.
- **Formulary:** List of prescription drugs covered by a plan.
- **Formulary Exclusion:** A drug omitted from coverage entirely, rather than placed on a higher tier. Exclusion lists move during the plan year, and a contract that permits unilateral changes lets the PBM alter the benefit after it is priced.
- **Fully Insured Plan:** Health plan where an insurance carrier assumes financial risk for claims.
- **Functional Fiduciary:** Person who assumes fiduciary responsibilities through their actions, even if not named.

G

- **Gag Clauses:** Contractual provisions restricting the disclosure of pricing or quality information, prohibited under the CAA.
- **Generic Drug:** Medication with same active ingredients, strength, dosage form, and route of administration as a brand-name drug but marketed under its chemical name.
- **GPO (Group Purchasing Organization):** An entity that contracts with drug manufacturers on a PBM's behalf, frequently under common ownership. Because manufacturer payments can be routed to the GPO rather than the PBM, revenue defined as belonging to the GPO may fall outside a rebate definition that names only the PBM. Also called a rebate aggregator.
- **Grandfathered Plan:** Health plan in place before the ACA, exempt from certain ACA requirements.

H

- **HDHP (High-Deductible Health Plan):** Health insurance plan with lower premiums and higher deductibles than traditional plans, often paired with Health Savings Accounts (HSAs).
- **HIPAA (Health Insurance Portability and Accountability Act):** Federal law protecting the privacy and security of health information.

I

- **ICHRA (Individual Coverage Health Reimbursement Arrangement):** Type of benefit allowing employers to provide employees with tax-advantaged funds to purchase individual health insurance coverage or pay for qualified medical expenses.
- **In-Network Provider:** Healthcare provider who contracts with a plan to offer services at discounted rates.
- **Ingredient Cost:** The portion of the total drug cost that reflects the actual cost of the medication itself.

J

- **J-Code:** Medical billing code used to identify injectable drugs.
- **Kickback (Anti-Kickback Statute):** Federal prohibition on paying or receiving anything of value to induce referrals or purchases reimbursed by a federal healthcare program. Relevant to pharmacy benefits wherever formulary placement tracks manufacturer payments rather than clinical value.

L

- **Laser:** Practice used by stop-loss insurance carriers to exclude specific high-cost individuals or treatments from coverage under an employer's stop-loss policy.
- **LNC (Lowest Net Cost):** A pricing evaluation approach based on total cost after all rebates, fees, and discounts are considered.

M

- **MAC List (Maximum Allowable Cost List):** A list setting the maximum reimbursement for generic drugs and brand drugs with generic equivalents. MAC governs what the pharmacy is paid. It does not govern what the

plan is charged, and where a contract permits those two figures to differ, the gap is spread. Ask whether the plan and the pharmacy are priced off the same list.

- **MHPAEA (Mental Health Parity and Addiction Equity Act):** Law requiring equal coverage for mental health and substance use disorder benefits.

N

- **NADAC (National Average Drug Acquisition Cost):** A CMS-published benchmark reflecting the average invoice price pharmacies pay for medications.
- **Named Fiduciary:** Fiduciary explicitly identified in the plan document.

O

- **Out-of-Network Provider:** Healthcare provider not contracted with the plan, usually leading to higher participant costs.
- **Out-of-Pocket Maximum:** Maximum a participant pays during a plan year, after which the plan covers all covered costs.

P

- **Participant:** Individual enrolled in a plan and eligible to receive benefits.
- **Pass-Through Pricing:** A model in which money remains a plan asset. The plan pays the pharmacy's actual price, receives every manufacturer dollar, and pays the PBM only the administrative fee the contract names. It is not pass-through if the PBM retains a spread on price, a share of rebates, or other manufacturer compensation.
- **PBM (Pharmacy Benefit Manager):** Administers prescription drug programs for health plans.
- **PHI (Protected Health Information):** Individually identifiable health information.
- **Plan Administrator:** Manages plan operations and compliance.
- **Plan Document:** Governing instrument detailing a plan's operations.
- **Plan Sponsor:** Employer or entity that establishes and maintains a plan.
- **PPO (Preferred Provider Organization):** Plan offering a network of providers at negotiated rates.
- **Prior Authorization:** A requirement that the PBM approve a drug before the plan will cover it. A utilization management tool that can control cost or obstruct access, depending on how the criteria are set and who sets them.
- **Prudent Person Rule:** Fiduciaries must act with the care, skill, prudence, and diligence that a prudent person familiar with such matters would use. ERISA states the standard in terms of a prudent person, and courts have read it to require familiarity with the subject, which is why it is often called a prudent expert standard.

Q

- **Qualified Plan:** Plan meeting IRS requirements, offering tax benefits.

R

- **Rebate:** A payment from a drug manufacturer tied to formulary placement, market share, or volume. The contract definition decides everything: revenue excluded from the definition, such as administrative fees, market development funds, or performance payments, does not reach the plan even under a contract promising full rebate pass-through.
- **Rebate Retention:** When PBMs retain a portion of manufacturer rebates instead of passing through 100% to the plan sponsor.
- **Revenue Leakage:** Whether the contract closes every channel through which plan dollars could reach the PBM or its affiliates, not only the channels regulated today. Silence on a revenue channel is not neutral. It means the PBM keeps it.
- **Run-Out Period:** Time allowed after a plan year for submitting and paying claims.

S

- **Self-Insured Plan:** Plan where the employer assumes financial risk for claims, often using stop-loss insurance.
- **Specialty Medication:** High-cost drug used to treat complex or chronic conditions. Often requires special handling, administration, or monitoring.
- **Specialty Pharmacy:** A pharmacy that dispenses high-cost or complex medications requiring special handling or monitoring.
- **Spread Pricing:** A pricing model where the PBM charges the plan sponsor more than it reimburses the pharmacy and keeps the difference.
- **Step Therapy:** A requirement that a participant try a preferred drug before a more expensive one is covered. Appropriate when driven by clinical evidence, and a cost-shifting device when driven by rebate economics.
- **Stop-Loss Insurance:** Protects self-insured employers from catastrophic claims.
- **Summary Plan Description (SPD):** Document summarizing plan benefits and participant rights.
- **Surprise Billing:** Unexpected out-of-network costs, addressed by No Surprises Act.

T

- **TPA (Third-Party Administrator):** Company providing administrative services for health plans.
- **Transparency:** Whether the plan knows where every dollar goes and can prove it. It has two halves the market routinely collapses into one. Disclosure means the vendor shows every dollar paid and retained. Verification means the plan can check those figures itself, on data it owns, through an audit it controls. A number the plan cannot audit, on data it does not own, is not transparent.
- **Transparency in Coverage Rule:** Requires health plans to publish negotiated rates and out-of-network amounts in machine-readable files, and to give participants cost-sharing estimates before care.
- **Trustee:** A fiduciary responsible for managing plan assets.

U

- **Utilization Management:** The set of controls governing access to a drug, including prior authorization, step therapy, and quantity limits. Where the contract lets the PBM change these unilaterally, the benefit can be altered after the plan has priced it.

V

- **Vertical Integration:** Common ownership of a PBM alongside an insurer, a specialty or mail-order pharmacy, a provider group, or a group purchasing organization. Integration lets margin move between affiliates, so a contract that constrains only the PBM entity may leave the affiliates untouched.

W

- **WAC (Wholesale Acquisition Cost):** A manufacturer's published list price to wholesalers, before discounts or rebates. A starting point for negotiation rather than a price anyone actually pays.
- **White Bagging:** Shipping a specialty drug from a contracted pharmacy directly to the site of care, instead of the provider buying and billing for it. Reduces provider markup, and some provider contracts restrict it.

Words matter, especially when they appear in your contract. Every clause, definition, and "industry standard" carries financial and legal consequences. Use this glossary to challenge vague terms, spot hidden conflicts, and demand clarity.

In a world of complex pharmacy benefits, plain language is a fiduciary weapon.

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